

Station 1:

1) A 36 year old women presents for a missed menses. She admits to having had unprotected intercourse with a new partner. She has positive pregnancy test. She already has 2 children and did not intend to have any more. She has uncontrolled diabetes and does not want to continue this pregnancy as it would be very risky. She is a single mother and is barely able to financially support herself and her kids. Based on LMP, you confirm she is <10 weeks along. Which of the following is not a mechanism of action of the medications you prescribe to induce an abortion?

- A.** Stimulates myometrial contraction
- B.** Thickens cervical mucous
- C.** Promotes cervical ripening
- D.** Competitive progesterone-receptor antagonist

2) Additionally, you discuss the importance of screening for sexually transmitted infections. The patient denies any symptoms, but she has had Chlamydia in the past. She is not aware of any infections her new partner may have had, but admits they are not monogamous with each other. She also reports her 2 children have different fathers. What sexually transmitted infections should she be screened for?

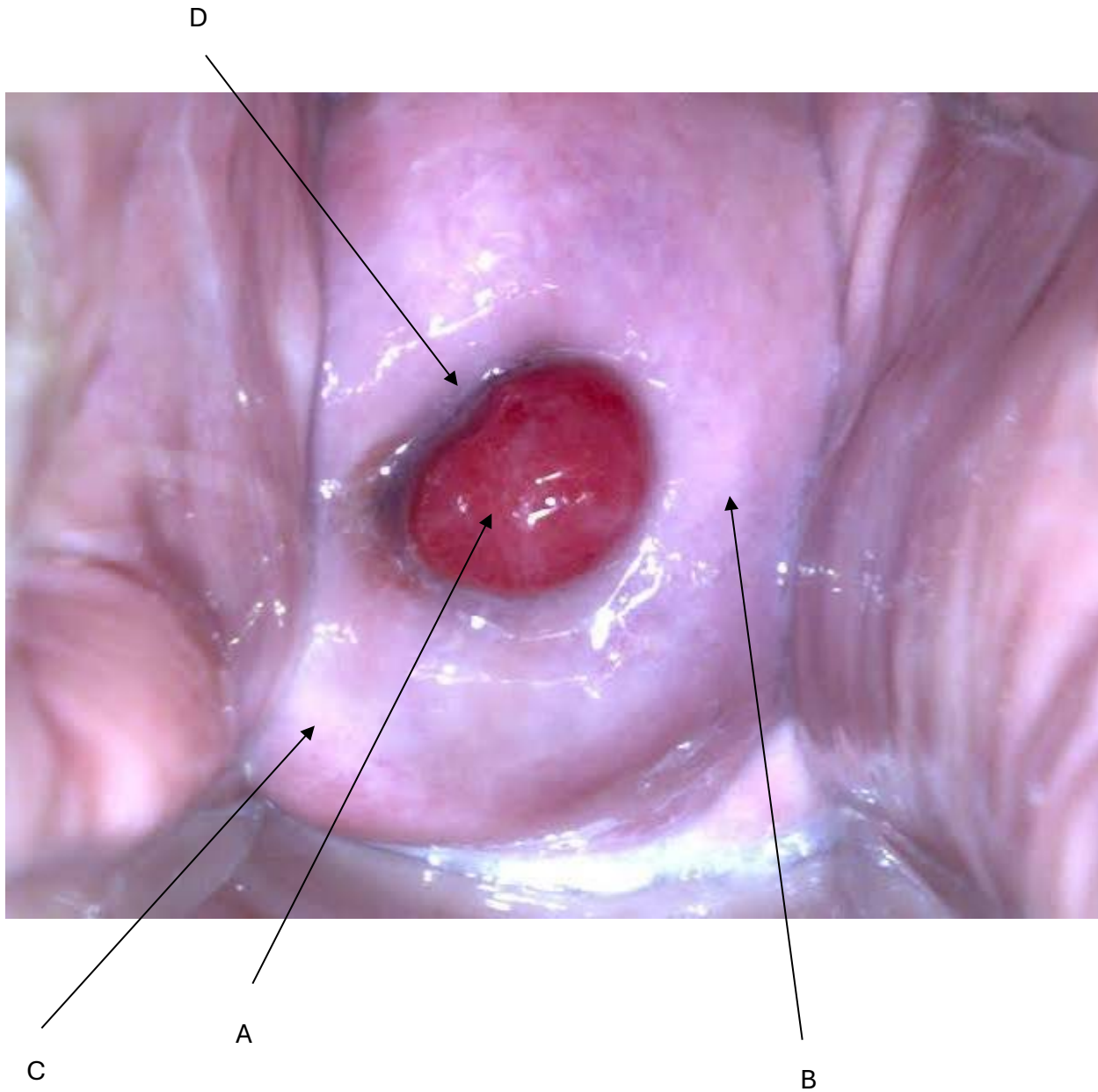
- A.** Candida albicans
- B.** Herpes Simplex Virus
- C.** Bacterial Vaginosis
- D.** Chlamydia trachomatis

3) She returns a few weeks later after a successful abortion and needs a preventative examination. Her Chlamydia test came back positive and she was appropriately treated. Past surgical history includes are 2 c-sections. She has a family history of colon cancer in her father at age 56. Mother has diabetes and hypertension. She is on metformin and empagliflozin currently and has no drug allergies. She drinks 1 alcoholic beverage a month on average, smokes ½ ppd, denies illicit drug use. She does not want any more children. What form of contraception is contraindicated?

4) On examination, her BMI is 29, BP is 130/84 mmHg, HR 79, normal temperature and respirations. She reports since her pregnancies, her menses have gotten heavier, lasting 7 days. She has had to miss work on occasion due to heavy flow and cramps. Which contraception would be most appropriate?

- A.** Nexplanon
- B.** Copper intrauterine device (IUD)
- C.** Levonorgestrel intrauterine device (IUD)
- D.** Medroxyprogesterone injections

- 5) You perform a pelvic examination and find normal external female genitalia without sores or ulcerations. Upon separation of the labia, you note the vaginal walls appear moist without excessive discharge. A speculum is inserted and you see what is shown in the picture below. **Please identify the item labeled A in the picture below (it is red, oval in shape)**

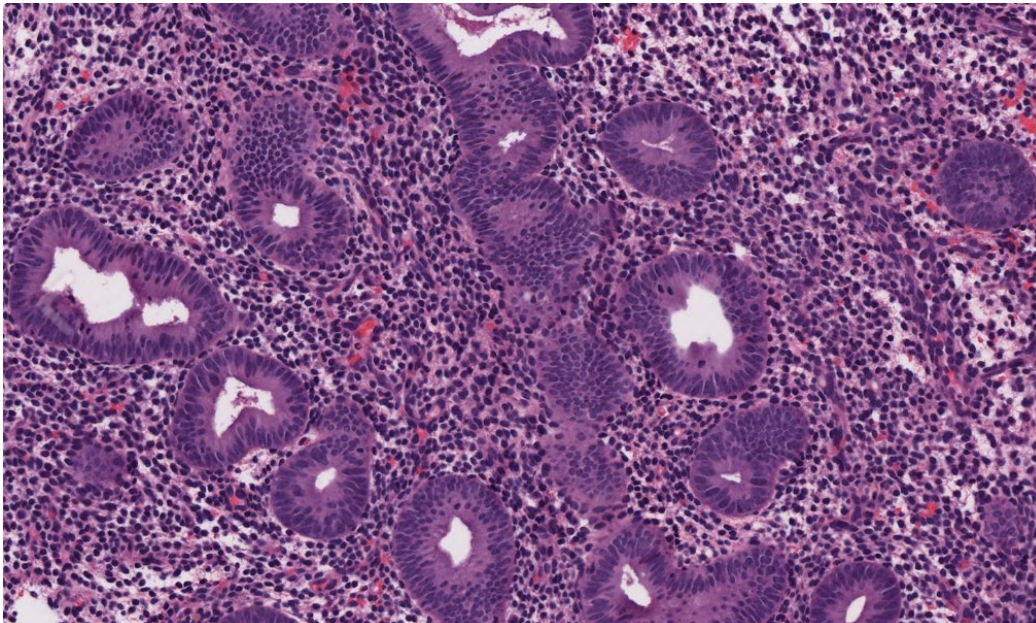


Station 2:

1) A 35-year-old female has been diagnosed with large leiomyomas. She is opting for medical management with leuprolide. The receptors for this medication are located at which part of the reproductive system axis?

- A. Hypothalamus
- B. Pituitary
- C. Ovary
- D. Myometrium

2)



Which of the following best explains the histological features observed in this endometrial biopsy?

- A. Progesterone-induced glandular secretion and stromal edema
- B. Estrogen-driven proliferation of the functional zone
- C. LH surge-induced follicular rupture and luteinization
- D. FSH-mediated recruitment of multiple tertiary follicles
- E. Decline in estrogen and progesterone triggering spiral artery constriction

3) Use the following 3D printed models to match the gross pathological features with their histology; one of the histology slides can be used twice. Write the name of the pathological condition.

- A
- B
- C
- D
- E
- F

4) A 45-year-old woman presents with heavy menstrual bleeding and intermittent cramping. She reports passing large clots during her period and experiencing fatigue due to anemia. Pelvic ultrasound reveals a hypoechoic mass projecting into the endometrial cavity.

Which of the 3D pathologies corresponds to this patient's condition?

- A
- B
- C
- D
- E
- F

5) A 55-year-old woman presents with painless vaginal bleeding that has occurred intermittently over the past two months. Pelvic ultrasound reveals a large mass filling the endometrial cavity. Cytogenetic studies of the mass revealed a mutation in the PTEN gene.

Which one of the following 3D pathologies represents this patient's condition?

- A
- B
- C
- D
- E
- F

Station 3:

- 1) A histological section of the ovary from a 28-year-old woman is shown. **Which of the following best describes the primary function of the structure marked by the arrow?**



- A. Estrogen production in response to FSH
- B. Progesterone secretion to maintain the endometrium
- C. Oocyte maturation and release
- D. Inhibition of LH via negative feedback
- E. Recruitment of primordial follicles

- 2) A patient with oligo-ovulation is looking to get pregnant. She is prescribed clomiphene to help with ovulation. **What is the mechanism of action of this medication? Select all that apply**

- A. Stimulates estrogen receptors in the pituitary
- B. Blocks estrogen receptors in the pituitary
- C. Stimulates estrogen receptors in the hypothalamus
- D. Blocks estrogen receptors in the hypothalamus

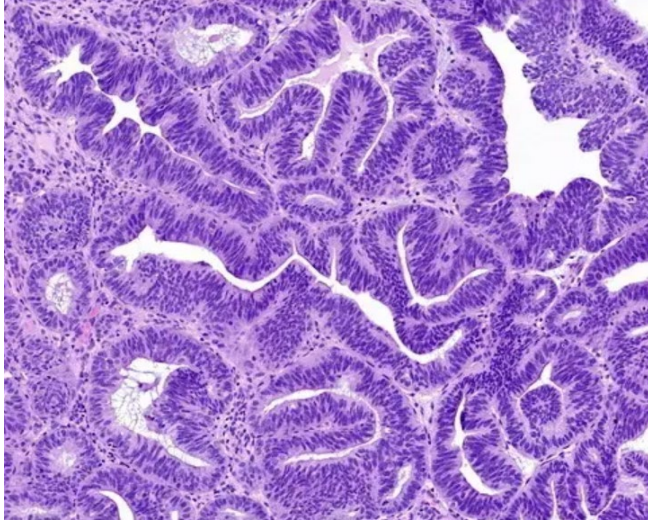
3)



Fill in the following table regarding the likely hormonal changes found in a patient with these ovaries versus normal hormonal profile (increase/decrease/same)

Testosterone	LH	FSH	Estrogen	Insulin resistance

4) A 55-year-old woman presents with vaginal bleeding. Pelvic ultrasound reveals a left ovarian mass. A photomicrograph of an endometrial biopsy is provided.



Which of the following is the most likely diagnosis?

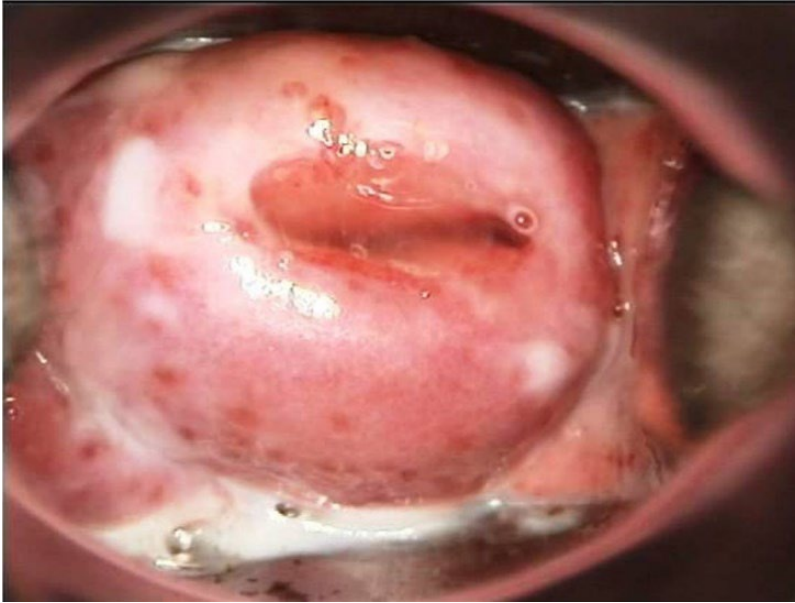
- A. Granulosa cell tumor
- B. Sertoli-Leydig cell tumor
- C. Serous cystadenocarcinoma
- D. Immature teratoma
- E. Dysgerminoma

5- Fill in the spaces:

The ovarian artery and vein are found within a peritoneal ligament named _____ as they travel to and from the ovary, the left ovarian vein drains into _____ while the left ovarian artery is a branch from _____.

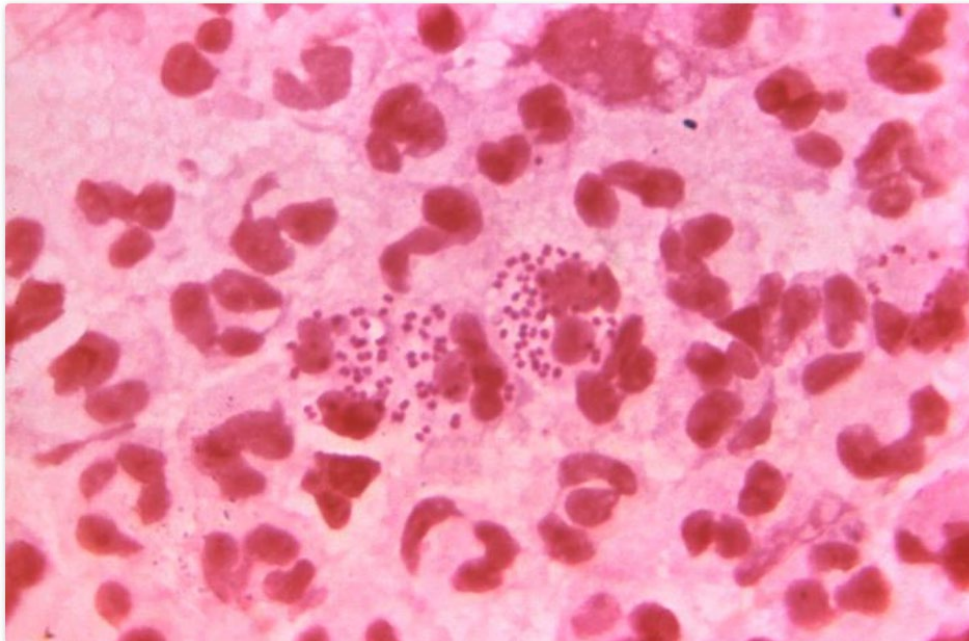
Station 4:

1) What is the appropriate treatment for a patient with this physical exam finding?



- A. Fluconazole
- B. Metronidazole
- C. Azithromycin
- D. Doxycycline

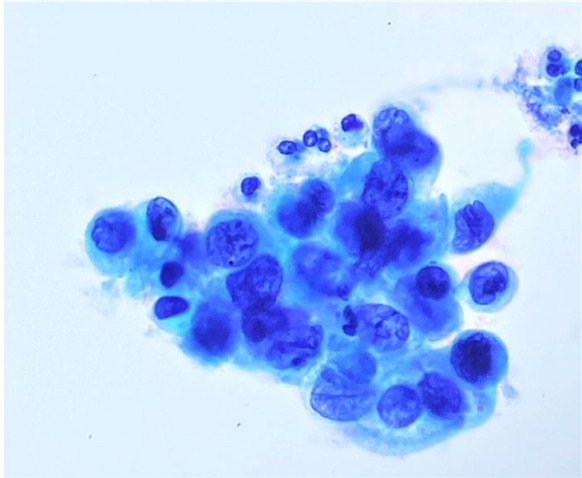
2) A patient presents with vaginal discharge. The following is seen on gram stain.



What is the most likely diagnosis?

- A. Chlamydia trachomatis
- B. Staphylococcus aureus
- C. Neisseria gonorrhea
- D. Gardnerella vaginitis

3) A 32-year-old woman presents for routine cervical cancer screening. Her Pap smear is shown.



Which one of the following histopathology findings is highly likely if a colposcopic biopsy is performed?
Select whatever applies.

- A
- B
- C
- D
- E

4) Which one of the following statements is most accurate regarding the pathogenesis of this patient's condition?

- A. HPV types 6 and 11 integrate into host DNA and inactivate p53
- B. HPV types 16 and 18 produce E6 and E7 proteins that inactivate tumor suppressors
- C. HPV infects the endocervical canal, leading to squamous metaplasia and dysplasia
- D. Condyloma acuminata is a precursor lesion for high-grade cervical intraepithelial neoplasia
- E. The transformation zone is resistant to HPV infection due to its columnar epithelium

5) A 41 year old female with a history of HIV comes in for a preventative exam. Her CD4 count is zero and she has been stable on antiviral therapy. She is single and sexually active intermittently, but not monogously. Her cervical cancer screening was last year and showed normal cytology with negative HPV testing. **Which of the following is not an accurate recommendation based on the current screening guidelines?**

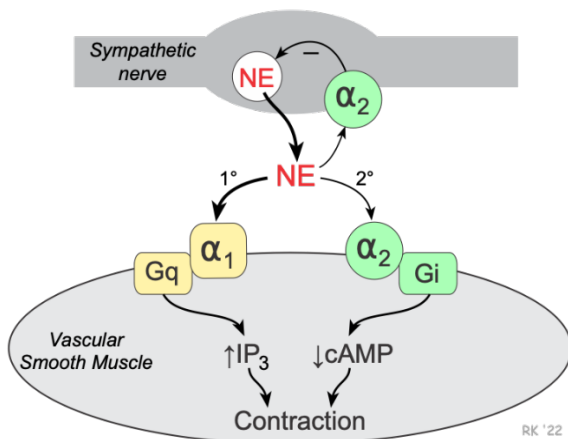
- A. She should receive her next pap smear and HPV test in 4 years as they are recommended every 5 years for this patient
- B. She should be offered a DoxyPEP prescription of Doxycycline 200 mg to be taken within 24-72 hours after intercourse
- C. She should receive an anal pap smear for screening
- D. She should be screened for other sexually transmitted infections like Chlamydia and Gonorrhea

Station 5:

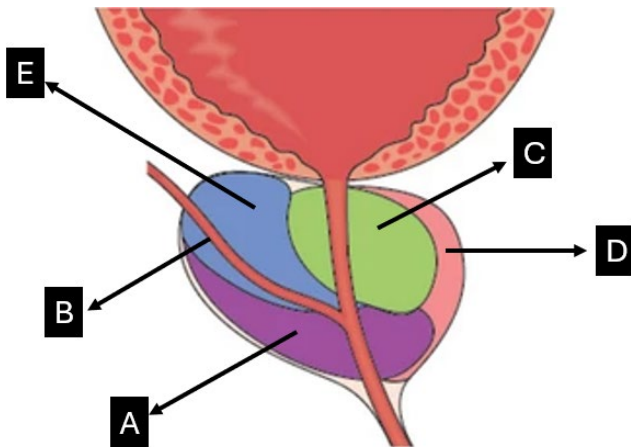
1) A patient is started on a medication for BPH. Which of the following is a possible side effect from this medication?

- A. Appetite suppression
- B. Smooth muscle contraction of the bladder neck
- C. Hypertension
- D. Pupil constriction

2) Which one of the following is the receptor for this medication?



3) Fill in the spaces based on the figure below: __ is most likely affected by PCA. __ is formed by the union of the vas deferens (ductus deferens) and the duct of the seminal vesicle.

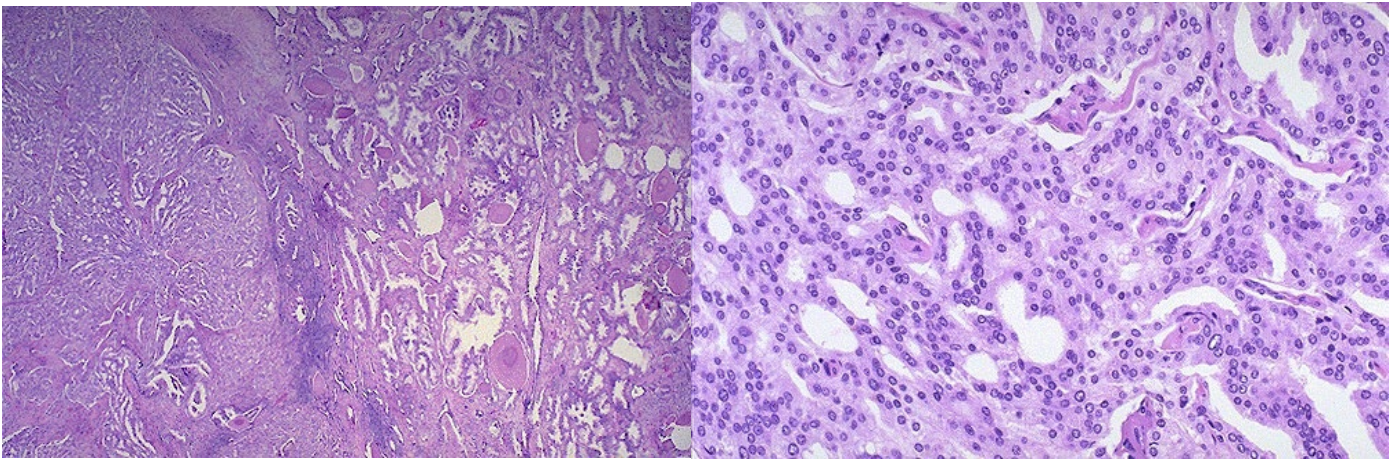


4) Pathology specimens from two patients were mixed up. By looking at the patients' clinical history:

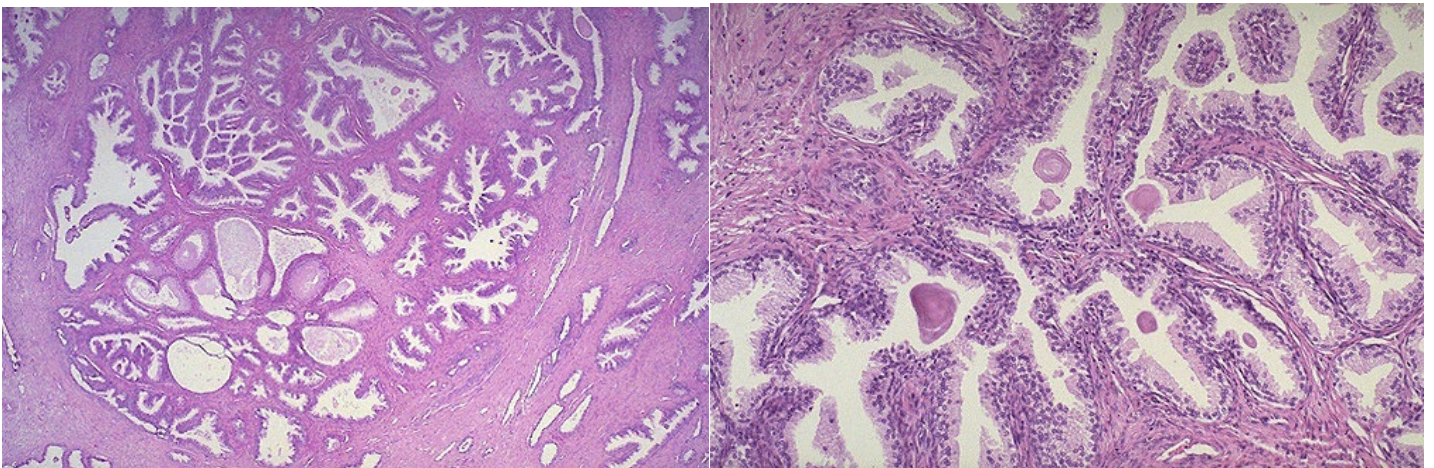
- **Patient A:** A 68-year-old man reports difficulty starting urination and frequent nighttime awakenings to void.
- **Patient B:** A 72-year-old man presents with new lower back pain.

Match the following histopathology slide sets (low and high powers) with the right patient.

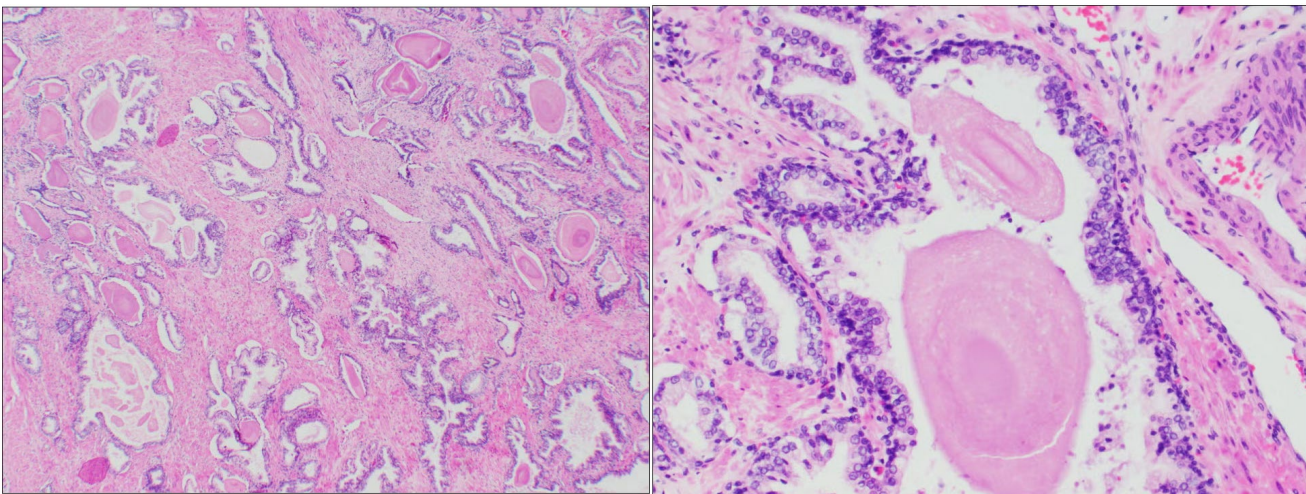
1.



2.



3.

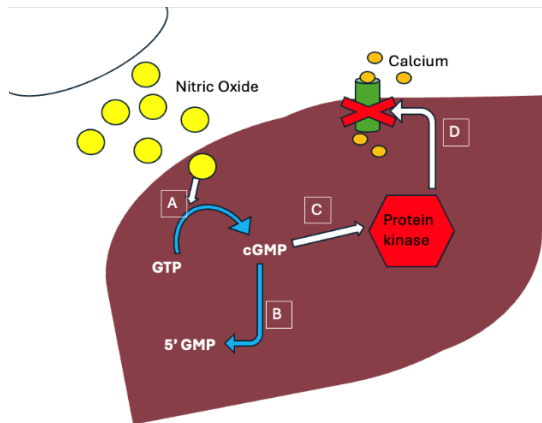


5) Which of the following best explains the patient's new onset of low back pain?

- A. Osteoblastic bone metastases causing increased bone formation
- B. Osteoclastic activity resulting in lytic bone lesions
- C. Degenerative changes associated with aging (osteoarthritis)
- D. Compression of the urethra due to periurethral nodular hyperplasia

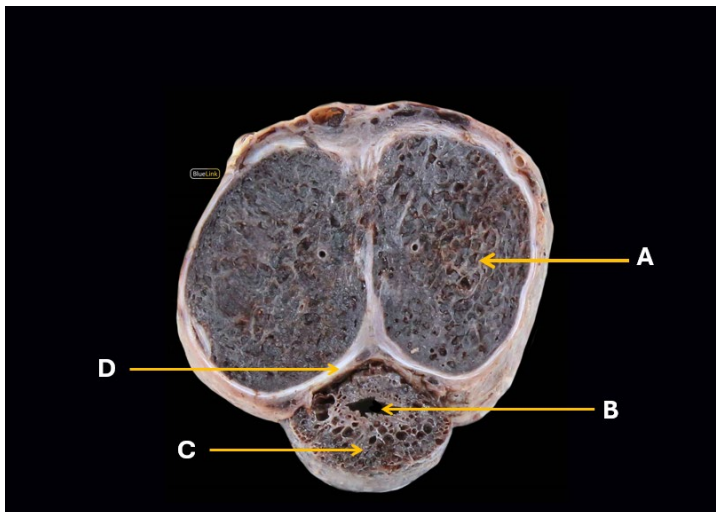
Station 6:

- 1) The molecular mechanism of an erection is shown below. **Which part of this process is directly inhibited by this Sildenafil?**



- A. A
B. B
C. C
D. D

- 2) A 52-year-old man presents with painful erections and difficulty with intercourse. Physical exam reveals abnormal curvature during erection. Fibrosis of which structure in this diagram is responsible for this patient's condition?

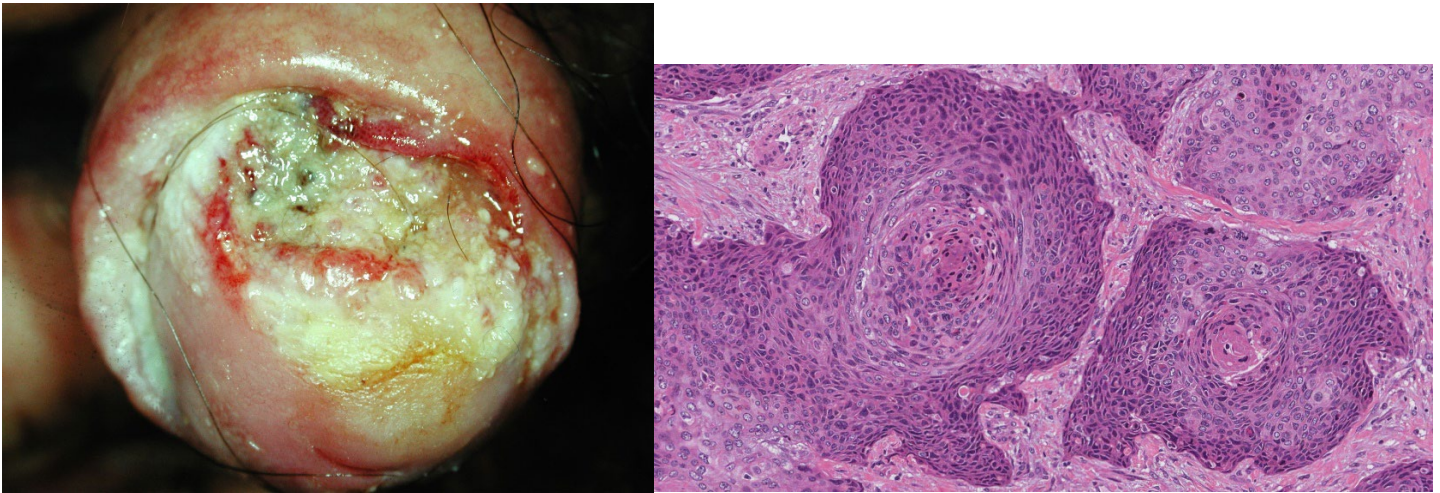


- 3) A 1-year-old male infant is admitted to the pediatric clinic because he is passing urine on the underside of the penis, as shown in the figure. **Which embryologic structure failed to fuse?**



- A. Spongy urethra
B. Labioscrotal folds
C. Urethral folds
D. Urogenital folds
E. Genital tubercle

- 4) A 58-year-old man presents with a slowly enlarging painless lesion on the glans penis for several months. Gross and histopathology images are provided for the lesion.



Which of the following statements is most accurate about this patient's condition?

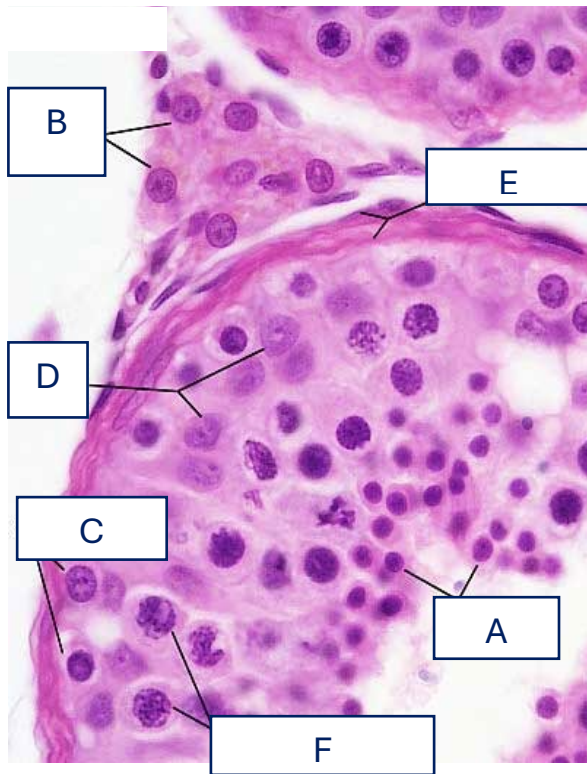
- A. HPV types 6 and 11 are the most likely cause
 - B. HPV types 16 and 18 are involved in pathogenesis
 - C. The HPV infection causes benign epithelial proliferation
 - D. The lesion typically regresses spontaneously without treatment
- 5) A 30-year-old man presents with a 5-day history of a painful genital ulcer. He reports recent unprotected sexual contact while traveling in a region where chancroid is prevalent. On examination, there is a single, deep, non-indurated ulcer with a purulent base and undermined edges on the shaft of the penis. He also has unilateral, tender inguinal lymphadenopathy. Syphilis serology and HSV NAAT are negative.

Which of the following features most reliably distinguishes this condition from primary syphilis and genital herpes?

- A. Painful ulcer with tender, unilateral inguinal lymphadenopathy
- B. Painless, indurated ulcer with non-tender lymphadenopathy
- C. Multiple, shallow, painful vesicular ulcers with bilateral lymphadenopathy
- D. Beefy-red, painless ulcer with no lymphadenopathy

Station 7:

1)



Which of the following cells are responsible for producing inhibin?

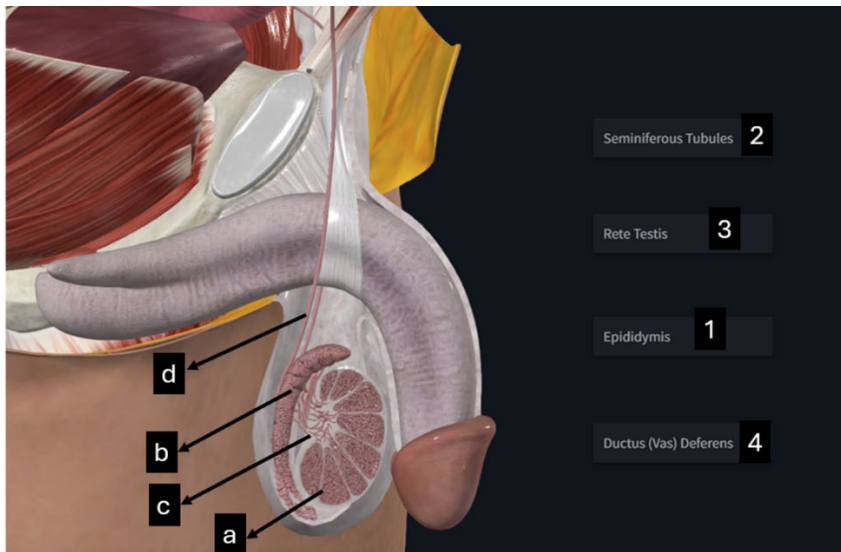
- A. A
- B. B
- C. C
- D. D

2) A 22-year-old man presents for evaluation of infertility. Physical examination is unremarkable. Laboratory studies reveal normal serum testosterone, but low sperm count. Further testing shows low serum follicle-stimulating hormone (FSH) and luteinizing hormone (LH). Testicular biopsy demonstrates normal numbers of Leydig cells and Sertoli cells.

Which of the following best describes the roles of LH and FSH in spermatogenesis and the specific testicular cells that mediate their effects?

- A. LH stimulates Leydig cells to produce testosterone, which is essential for spermatogenesis; FSH acts on Sertoli cells to support germ cell maturation.
- B. LH stimulates Sertoli cells to produce inhibin; FSH acts on Leydig cells to produce testosterone.
- C. LH and FSH both act directly on germ cells to promote spermatogenesis.
- D. LH stimulates peritubular myoid cells to produce androgen-binding protein; FSH acts on Leydig cells to produce testosterone.
- E. LH and FSH both act on the hypothalamus to increase GnRH secretion.

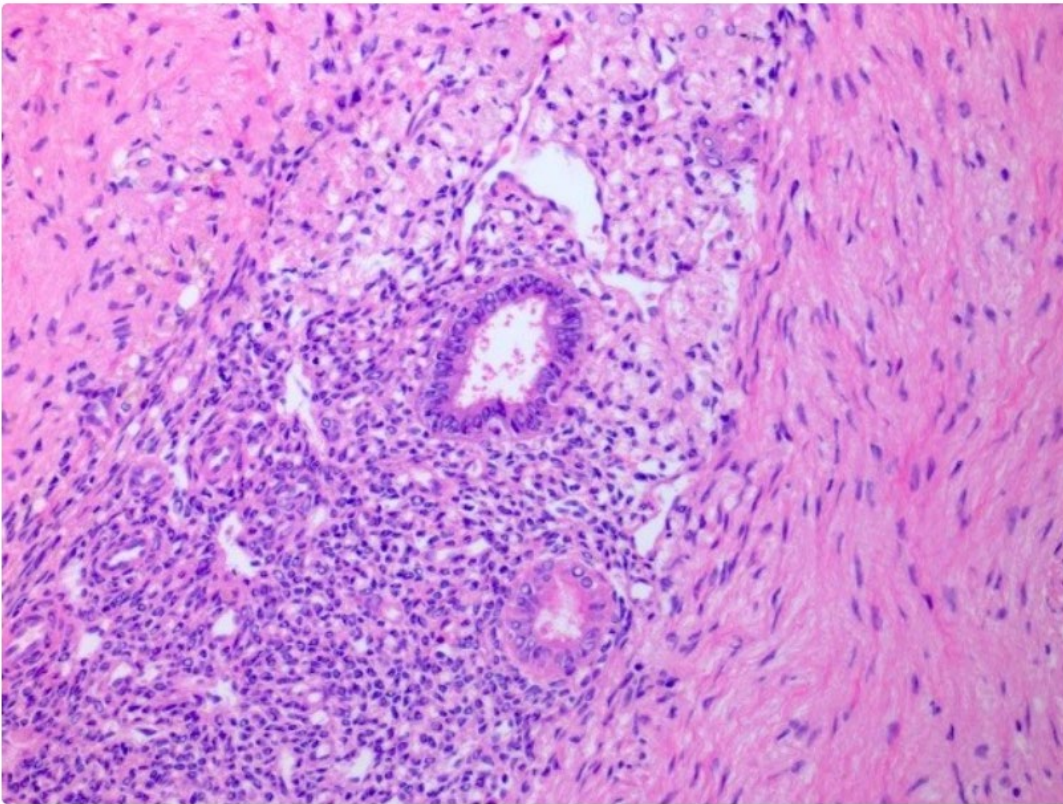
- 3) Match the structure name (numbers) with the anatomical structure (alphabet) and the provided histology (roman numbers)



- 4) A 4-year-old boy is brought to the physician by his mother because of a swelling in the abdomen. Physical examination shows a palpable mass located external to the aponeurosis of the external abdominal oblique and a nonpalpable right testis. Ultrasonography of the abdomen shows that the mass is an ectopic testis, classified as interstitial. **Which of the following embryological structures most likely failed to develop, resulting in the clinical presentation?**
- A. Gubernaculum
 - B. Processus vaginalis
 - C. Genital tubercle
 - D. Seminiferous cords
 - E. Labioscrotal swellings
- 5) Three years later, the same boy in the previous scenario was brought to his pediatrician because of early development of pubic hair, acne, and rapid growth. Physical exam shows asymmetry of the scrotum with a firm, non-tender mass in the left testis. Serum studies show elevated testosterone but normal α -fetoprotein (AFP) and human chorionic gonadotropin (hCG). **Which of the following is the most likely diagnosis?**
- A. Seminoma
 - B. Embryonal carcinoma
 - C. Leydig cell tumor
 - D. Yolk sac tumor
 - E. Choriocarcinoma

Station 8:

- 1) A woman gives birth to a term infant via cesarean section and begins to hemorrhage. The obstetrician ligates bilateral uterine arteries. The bleeding improves and the uterus does not need to be removed. How is the uterus now receiving blood flow? Fill in the blank
- _____
- 2) A 24-year-old woman comes to the emergency department because of lower abdominal pain. A CT scan of the abdomen and pelvis shows an abnormal mass occupying the left adnexa in the pelvis. During the surgical procedure to remove the tumor, the ureter and the structures immediately medial to the ureter are identified. **Which of the following vascular structures crosses the ureter just lateral to the cervix of the uterus?**
- A. Middle rectal artery
 - B. Superior vesical artery
 - C. Internal pudendal vein
 - D. Uterine artery
 - E. Gonadal vein
- 3) A 16-year-old woman is brought to the emergency department with severe abdominal pain and fever. Laboratory examination is remarkable for an elevated white blood cell count and a positive test for pregnancy. Colpocentesis is performed to ascertain the presence of blood in the pelvis from a ruptured ectopic pregnancy. **Through which of the following structures does the needle need to be inserted to perform colpocentesis?**
- A. Through the perineal body into the vesicouterine space
 - B. Through the posterior fornix of the vagina into the rectouterine pouch
 - C. Through the anterior fornix into the endocervical canal
 - D. Through the introitus into the vestibular gland
 - E. Through the perineal membrane into the urogenital diaphragm
- 4) A 30-year-old woman has had recurring pelvic pain with dysuria and dysmenorrhea for the past 5 years. On pelvic examination, no abnormal findings were noted. Magnetic resonance imaging shows focal lesions in the posterior cul-de-sac along with adhesions. Histology of the lesion is provided.



Which of the following is the most likely diagnosis?

- A. Endometriosis
- B. Hydatidiform mole
- C. Acute salpingitis
- D. Subserosal leiomyoma
- E. Poorly differentiated adenocarcinoma

5) Assemble the branches of the internal iliac artery in the 3D printed pelvis model in relation to the pelvic organs.

Station 1:

6) A 36 year old women presents for a missed menses. She admits to having had unprotected intercourse with a new partner. She has positive pregnancy test. She already has 2 children and did not intend to have any more. She has uncontrolled diabetes and does not want to continue this pregnancy as it would be very risky. She is a single mother and is barely able to financially support herself and her kids. Based on LMP, you confirm she is <10 weeks along. Which of the following is not a mechanism of action of the medications you prescribe to induce an abortion?

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7) Additionally, you discuss the importance of screening for sexually transmitted infections. The patient denies any symptoms, but she has had Chlamydia in the past. She is not aware of any infections her new partner may have had, but admits they are not monogamous with each other. She also reports her 2 children have different fathers. What sexually transmitted infections should she be screened for?

- E. Candida albicans
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- H. Chlamydia trachomatis**

8) She returns a few weeks later after a successful abortion and needs a preventative examination. Her Chlamydia test came back positive and she was appropriately treated. Past surgical history includes are 2 c-sections. She has a family history of colon cancer in her father at age 56. Mother has diabetes and hypertension. She is on metformin and empagliflozin currently and has no drug allergies. She drinks 1 alcoholic beverage a month on average, smokes ½ ppd, denies illicit drug use. She does not want any more children. What form of contraception is contraindicated?

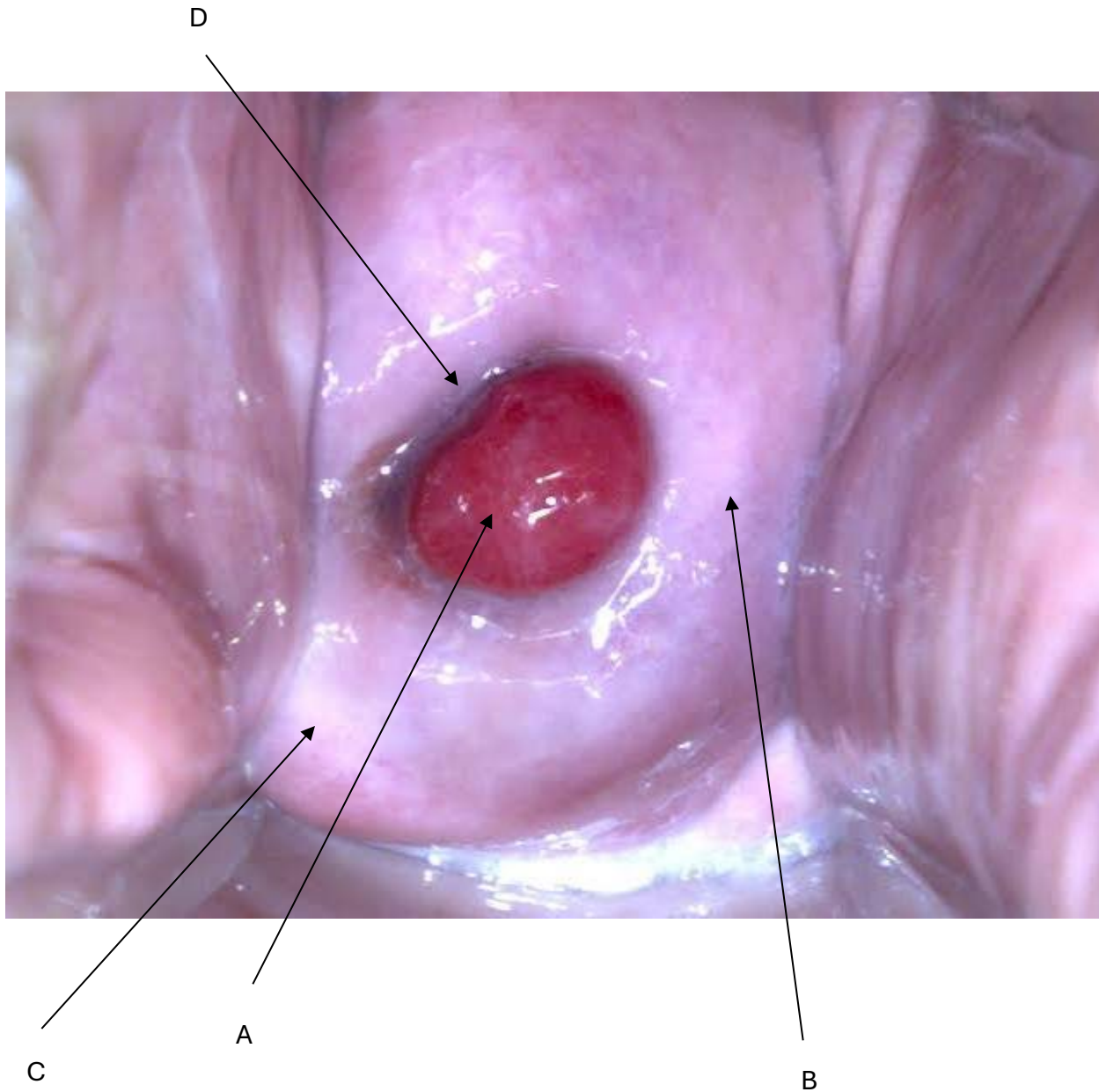
Estrogen-containing contraception (combined oral contraception, patch, or vaginal ring)

9) On examination, her BMI is 29, BP is 130/84 mmHg, HR 79, normal temperature and respirations. She reports since her pregnancies, her menses have gotten heavier, lasting 7 days. She has had to miss work on occasion due to heavy flow and cramps. Which contraception would be most appropriate?

- E. Nexplanon
- F. Copper intrauterine device (IUD)
- G. Levonorgestrel intrauterine device (IUD)**
- H. Medroxyprogesterone injections

10) You perform a pelvic examination and find normal external female genitalia without sores or ulcerations. Upon separation of the labia, you note the vaginal walls appear moist without excessive discharge. A speculum is inserted and you see what is shown in the picture below. **Please identify the item labeled A in the picture below (it is red, oval in shape)**

A is a Cervical polyp

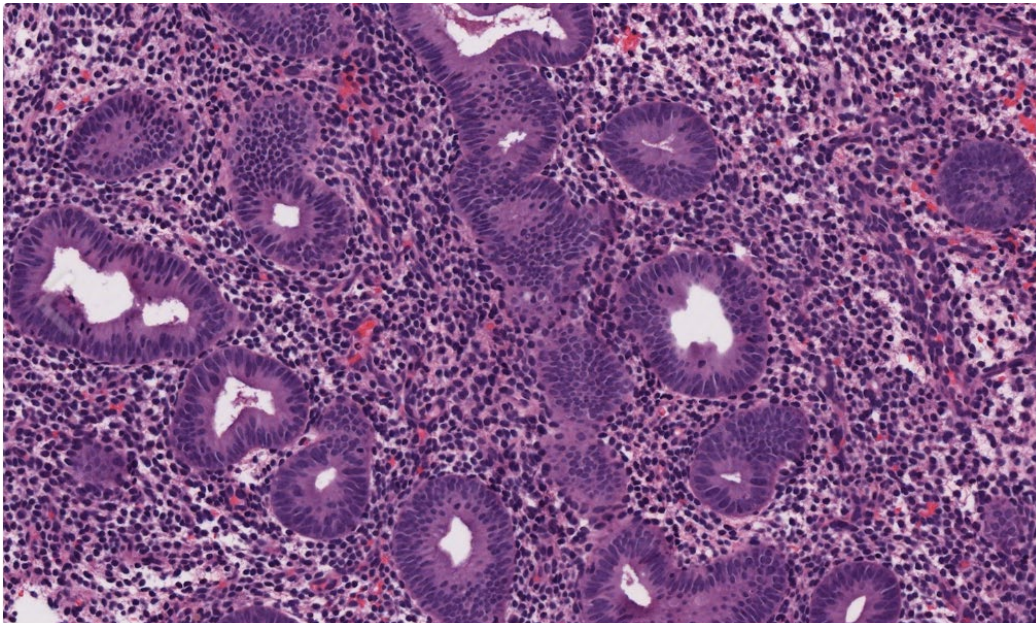


Station 2:

11) A 35-year-old female has been diagnosed with large leiomyomas. She is opting for medical management with leuprolide. The receptors for this medication are located at which part of the reproductive system axis?

- A. Hypothalamus
- B. Pituitary**
- C. Ovary
- D. Myometrium

12)



Which of the following best explains the histological features observed in this endometrial biopsy?

- F. Progesterone-induced glandular secretion and stromal edema
- G. Estrogen-driven proliferation of the functional zone**
- H. LH surge-induced follicular rupture and luteinization
- I. FSH-mediated recruitment of multiple tertiary follicles
- J. Decline in estrogen and progesterone triggering spiral artery constriction

3) Use the following 3D printed models to match the gross pathological features with their histology; one of the histology slides can be used twice. Write the name of the pathological condition.

- A, **6, Submucous fibroid**
- B, **6, Intramural and Subserosal Fibroid**
- C, **4, Endometrial polyp**
- D, **5, Endometrial carcinoma**
- E, **3, Cervical Cancer**
- F, **1, Adenomyosis**

2 is endometrial hyperplasia, and 7 is leiomyosarcoma.

4) A 45-year-old woman presents with heavy menstrual bleeding and intermittent cramping. She reports passing large clots during her period and experiencing fatigue due to anemia. Pelvic ultrasound reveals a hypoechoic mass projecting into the endometrial cavity.

Which of the 3D pathologies corresponds to this patient's condition?

A

B

C

D

E

F

5) A 55-year-old woman presents with painless vaginal bleeding that has occurred intermittently over the past two months. Pelvic ultrasound reveals a large mass filling the endometrial cavity. Cytogenetic studies of the mass revealed a mutation in the PTEN gene.

Which one of the following 3D pathologies represents this patient's condition?

A

B

C

D

E

F

Station 3:

- 2) A histological section of the ovary from a 28-year-old woman is shown. Which of the following best describes the primary function of the structure marked by the arrow?



- A. Estrogen production in response to FSH
- B. Progesterone secretion to maintain the endometrium**
- C. Oocyte maturation and release
- D. Inhibition of LH via negative feedback
- E. Recruitment of primordial follicles

- 4) A patient with oligo-ovulation is looking to get pregnant. She is prescribed clomiphene to help with ovulation. What is the mechanism of action of this medication? Select all that apply

- A. Stimulates estrogen receptors in the pituitary
- B. Blocks estrogen receptors in the pituitary**
- C. Stimulates estrogen receptors in the hypothalamus
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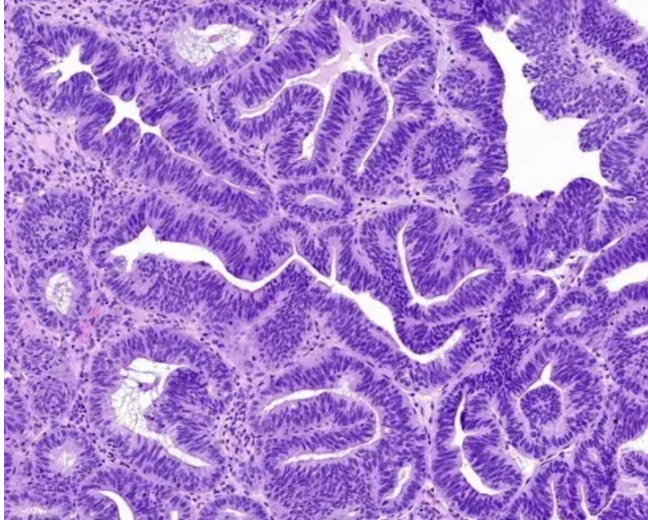
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Fill in the following table regarding the likely hormonal changes found in a patient with these ovaries versus normal hormonal profile (increase/decrease/same)

Testosterone	LH	FSH	Estrogen	Insulin resistance
increase	increase	decrease	decrease	increase

4) A 55-year-old woman presents with vaginal bleeding. Pelvic ultrasound reveals a left ovarian mass. A photomicrograph of an endometrial biopsy is provided.



Which of the following is the most likely diagnosis?

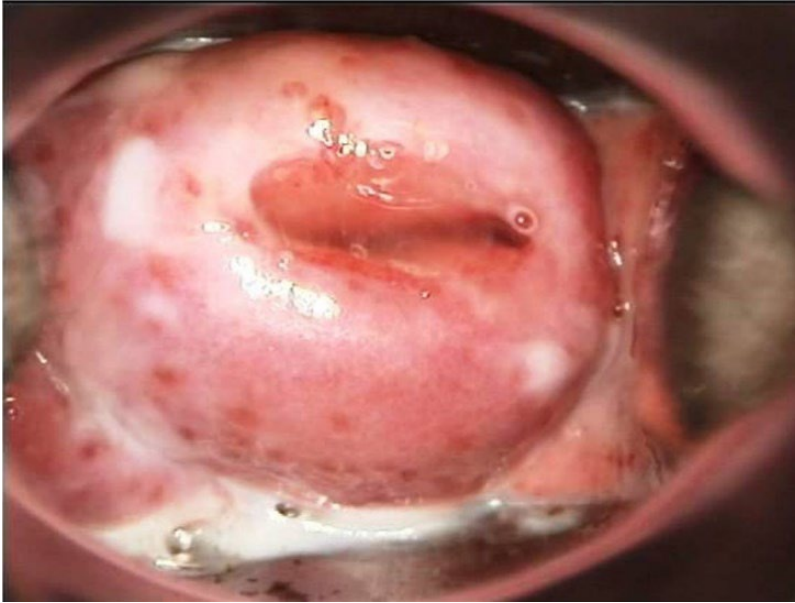
- A. Granulosa cell tumor
- B. Sertoli-Leydig cell tumor
- C. Serous cystadenocarcinoma
- D. Immature teratoma
- E. Dysgerminoma

5- Fill in the spaces:

The ovarian artery and vein are found within a peritoneal ligament named ____ Suspensory ligament of the ovary ____ as they travel to and from the ovary, the left ovarian vein drains into ____ left renal vein ____ while the left ovarian artery is a branch from ____ abdominal aorta ____.

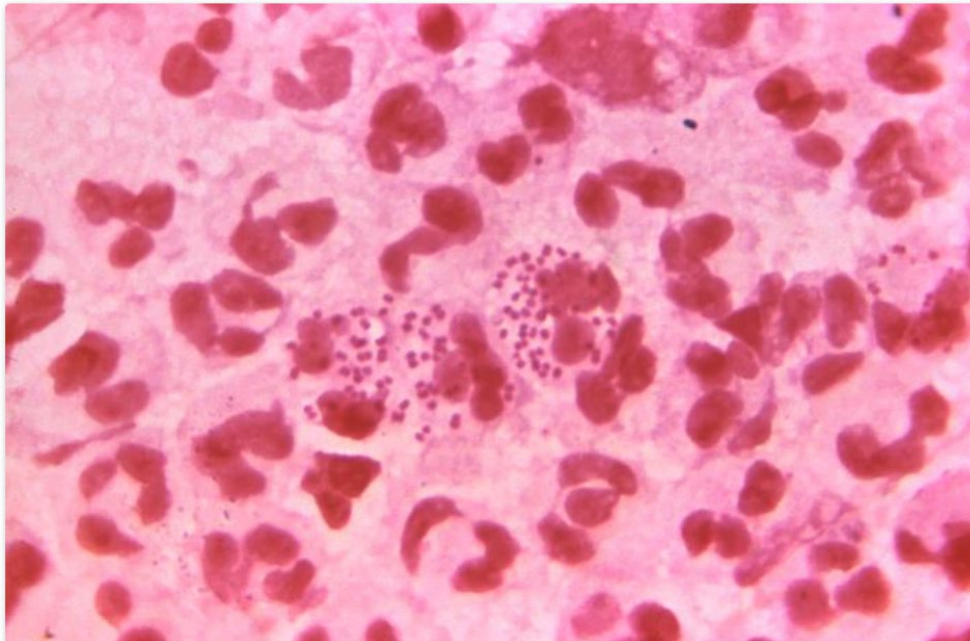
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5) What is the appropriate treatment for a patient with this physical exam finding?



- A. Fluconazole
- B. Metronidazole**
- C. Azithromycin
- D. Doxycycline

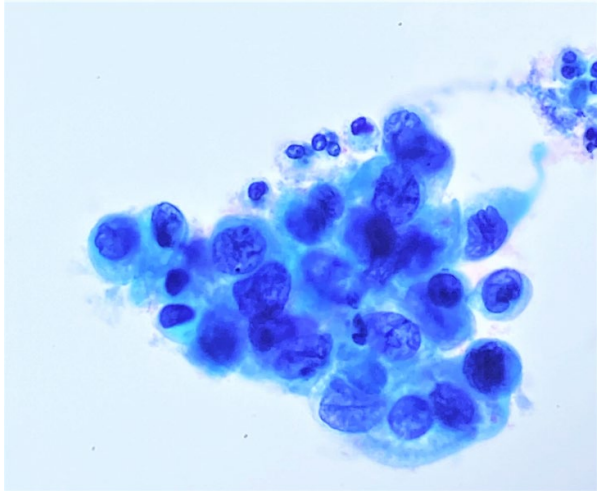
6) A patient presents with vaginal discharge. The following is seen on gram stain.



What is the most likely diagnosis?

- A. Chlamydia trachomatis
- B. Staphylococcus aureus
- C. Neisseria gonorrhea**
- D. Gardnerella vaginitis

7) A 32-year-old woman presents for routine cervical cancer screening. Her Pap smear is shown.



Which one of the following histopathology findings is highly likely if a colposcopic biopsy is performed?
Select whatever applies.

- A
- B
- C
- D
- E

- Normal, A
- CIN I, B
- CIN II, C
- CIN III, D
- Condyloma Acuminata, E

8) Which one of the following statements is most accurate regarding the pathogenesis of this patient's condition?

- F. HPV types 6 and 11 integrate into host DNA and inactivate p53
- G. HPV types 16 and 18 produce E6 and E7 proteins that inactivate tumor suppressors
- H. HPV infects the endocervical canal, leading to squamous metaplasia and dysplasia
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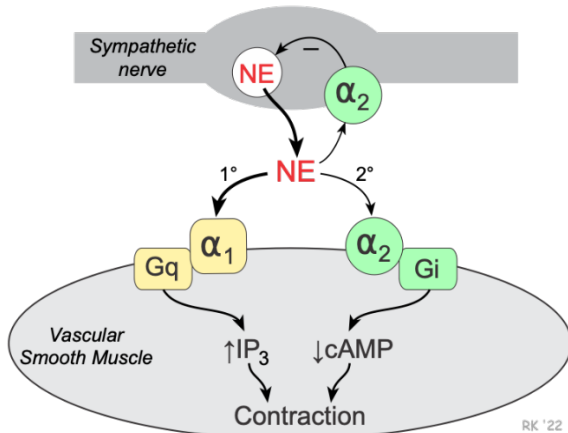
Station 5:

6) A patient is started on a medication for BPH. Which of the following is a possible side effect from this medication?

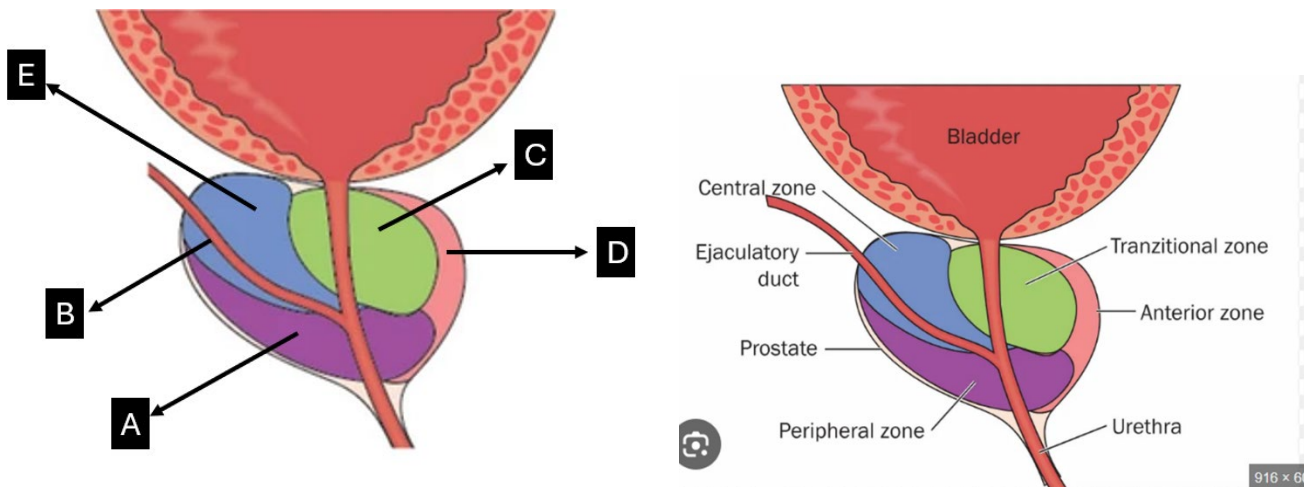
- A. Appetite suppression
- B. Smooth muscle contraction of the bladder neck
- C. Hypertension

D. Pupil constriction

7) Which one of the following is the receptor for this medication? **alpha-1 adrenergic receptor**



8) Fill in the spaces based on the figure below: A is most likely affected by PCA. B is formed by the union of the vas deferens (ductus deferens) and the duct of the seminal vesicle.

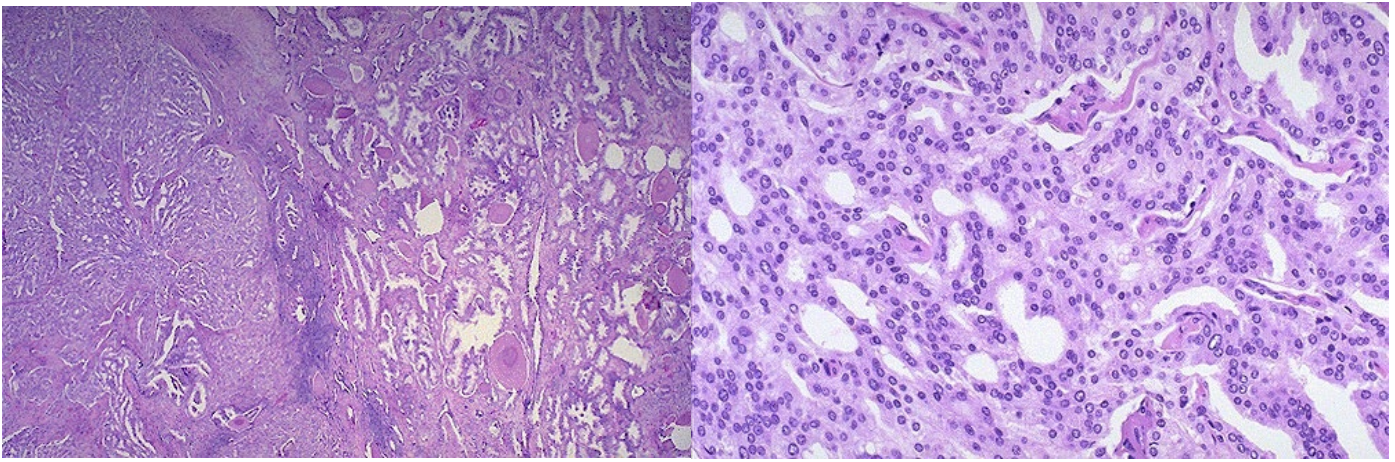


9) Pathology specimens from two patients were mixed up. By looking at the patients' clinical history:

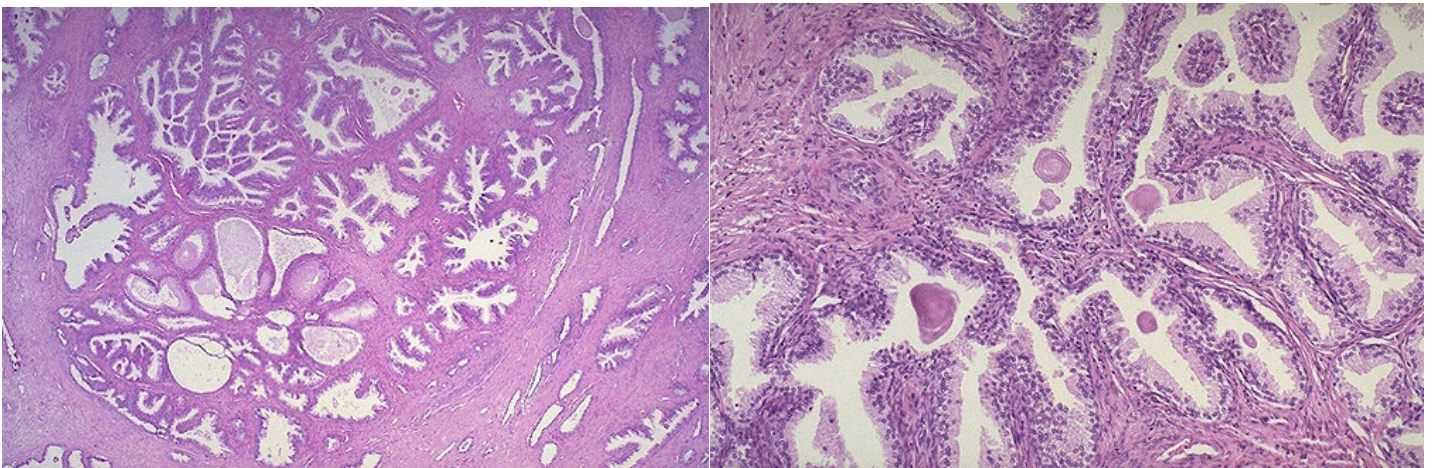
- **Patient A:** A 68-year-old man reports difficulty starting urination and frequent nighttime awakenings to void.
- **Patient B:** A 72-year-old man presents with new lower back pain.

Match the following histopathology slide sets (low and high powers) with the right patient.

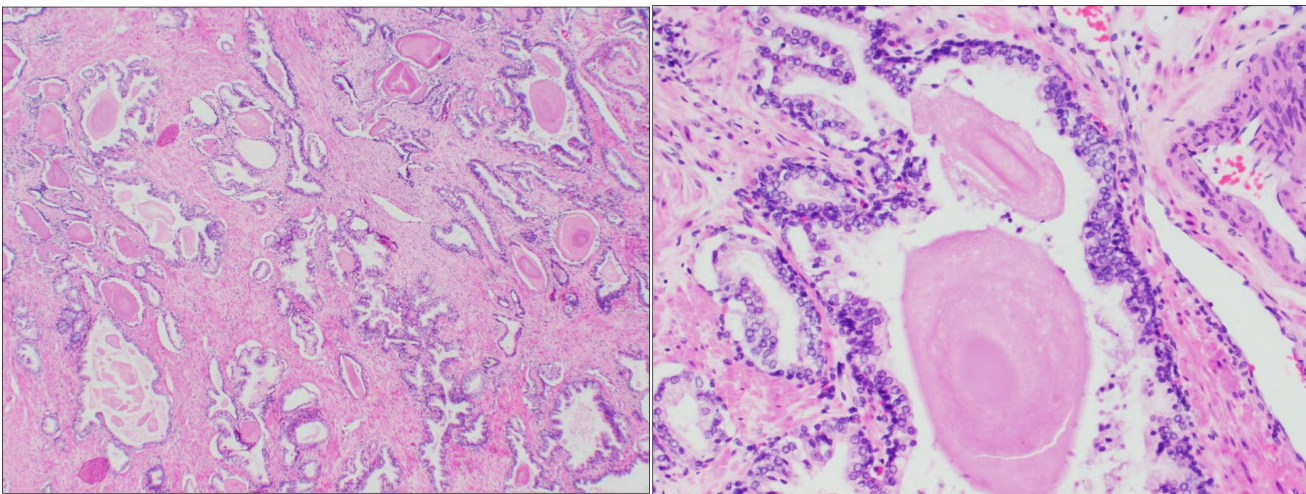
3. Patient B- PCA



4. Patient A-BPH



3. Normal Prostate

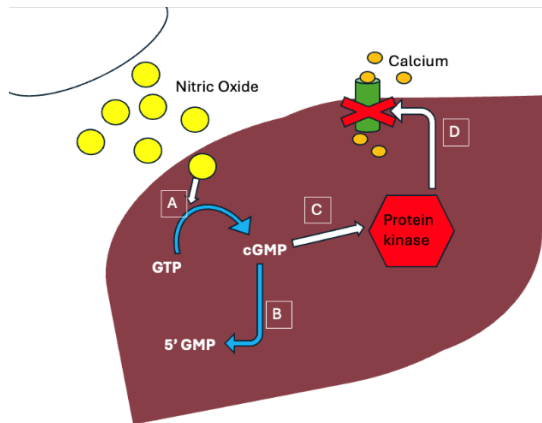


10) Which of the following best explains the patient's new onset of low back pain?

- A. Osteoblastic bone metastases causing increased bone formation
- B. Osteoclastic activity resulting in lytic bone lesions
- C. Degenerative changes associated with aging (osteoarthritis)
- D. Compression of the urethra due to periurethral nodular hyperplasia

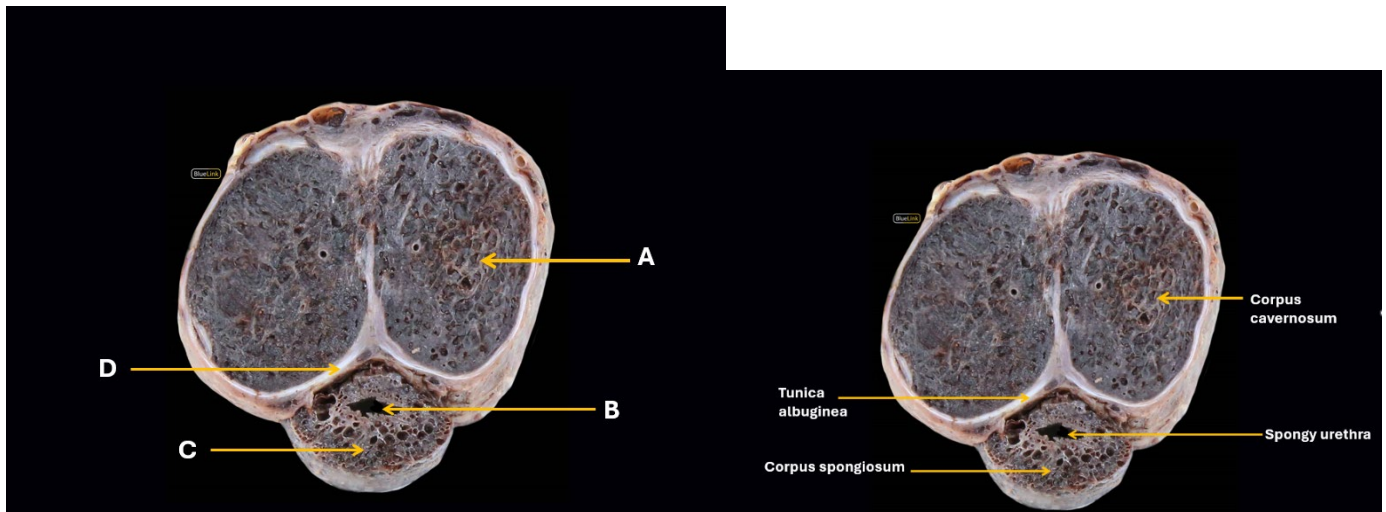
Station 6:

- 6) The molecular mechanism of an erection is shown below. Which part of this process is directly inhibited by this Sildenafil?



- E. A
F. B
G. C
H. D

- 7) A 52-year-old man presents with painful erections and difficulty with intercourse. Physical exam reveals abnormal curvature during erection. Fibrosis of which structure in this diagram is responsible for this patient's condition? D

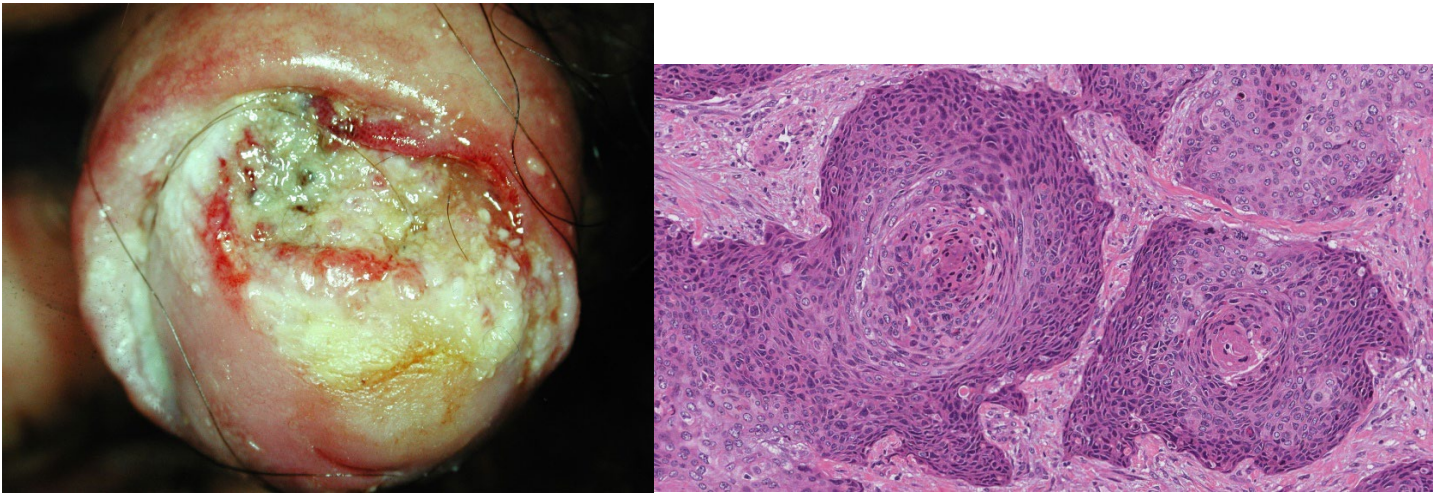


- 8) A 1-year-old male infant is admitted to the pediatric clinic because he is passing urine on the underside of the penis, as shown in the figure. Which embryologic structure failed to fuse?



- A. Spongy urethra
B. Labioscrotal folds
C. Urethral folds
D. Urogenital folds
E. Genital tubercle

- 9) A 58-year-old man presents with a slowly enlarging painless lesion on the glans penis for several months. Gross and histopathology images are provided for the lesion.



Which of the following statements is most accurate about this patient's condition?

- A. HPV types 6 and 11 are the most likely cause
- B. HPV types 16 and 18 are involved in pathogenesis**
- C. The HPV infection causes benign epithelial proliferation
- D. The lesion typically regresses spontaneously without treatment

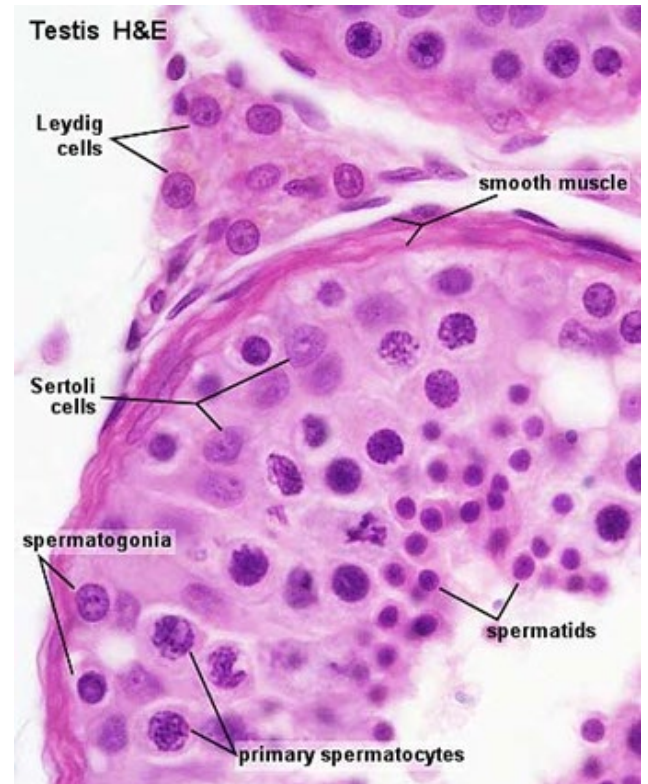
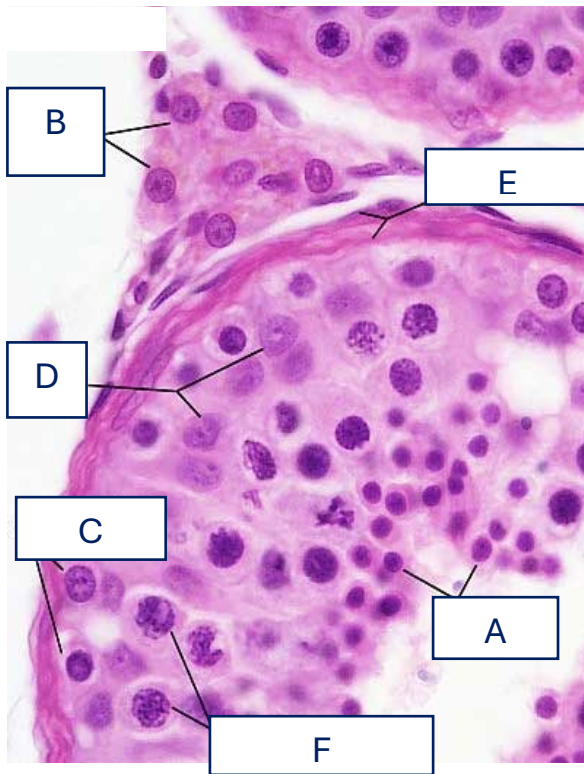
- 10) A 30-year-old man presents with a 5-day history of a painful genital ulcer. He reports recent unprotected sexual contact while traveling in a region where chancroid is prevalent. On examination, there is a single, deep, non-indurated ulcer with a purulent base and undermined edges on the shaft of the penis. He also has unilateral, tender inguinal lymphadenopathy. Syphilis serology and HSV NAAT are negative.

Which of the following features most reliably distinguishes this condition from primary syphilis and genital herpes?

- A. Painful ulcer with tender, unilateral inguinal lymphadenopathy**
- B. Painless, indurated ulcer with non-tender lymphadenopathy
- C. Multiple, shallow, painful vesicular ulcers with bilateral lymphadenopathy
- D. Beefy-red, painless ulcer with no lymphadenopathy

Station 7:

1)



Which of the following cells are responsible for producing inhibin?

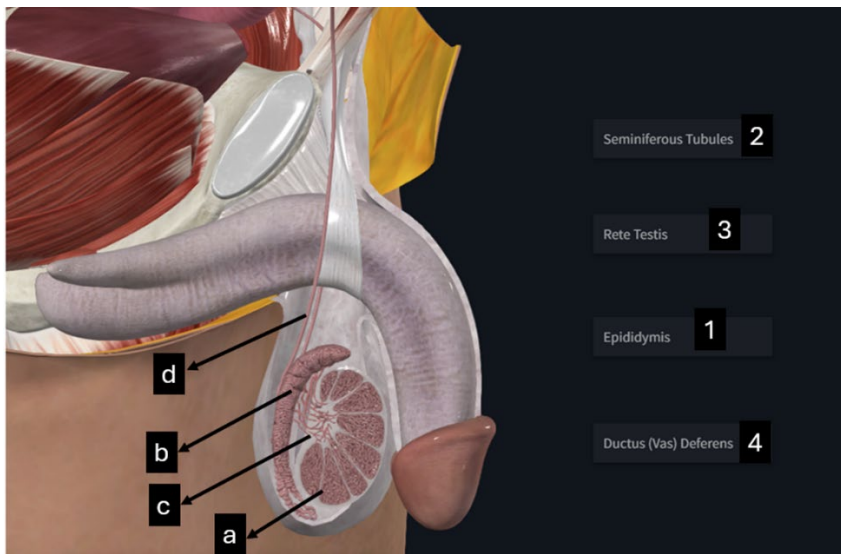
- A. A
- B. B
- C. C
- D. D**

6) A 22-year-old man presents for evaluation of infertility. Physical examination is unremarkable. Laboratory studies reveal normal serum testosterone, but low sperm count. Further testing shows low serum follicle-stimulating hormone (FSH) and luteinizing hormone (LH). Testicular biopsy demonstrates normal numbers of Leydig cells and Sertoli cells.

Which of the following best describes the roles of LH and FSH in spermatogenesis and the specific testicular cells that mediate their effects?

- A. LH stimulates Leydig cells to produce testosterone, which is essential for spermatogenesis; FSH acts on Sertoli cells to support germ cell maturation.**
- B. LH stimulates Sertoli cells to produce inhibin; FSH acts on Leydig cells to produce testosterone.
- C. LH and FSH both act directly on germ cells to promote spermatogenesis.
- D. LH stimulates peritubular myoid cells to produce androgen-binding protein; FSH acts on Leydig cells to produce testosterone.
- E. LH and FSH both act on the hypothalamus to increase GnRH secretion.

- 7) Match the structure name (numbers) with the anatomical structure (alphabet) and the provided histology (roman numbers)



- A, 2. seminiferous tubules, IV
- B, 1. Epididymis, II
- C, 3. Rete Testis, I
- D, 4. Ductus Deferens, III

- 8) A 4-year-old boy is brought to the physician by his mother because of a swelling in the abdomen. Physical examination shows a palpable mass located external to the aponeurosis of the external abdominal oblique and a nonpalpable right testis. Ultrasonography of the abdomen shows that the mass is an ectopic testis, classified as interstitial. **Which of the following embryological structures most likely failed to develop, resulting in the clinical presentation?**

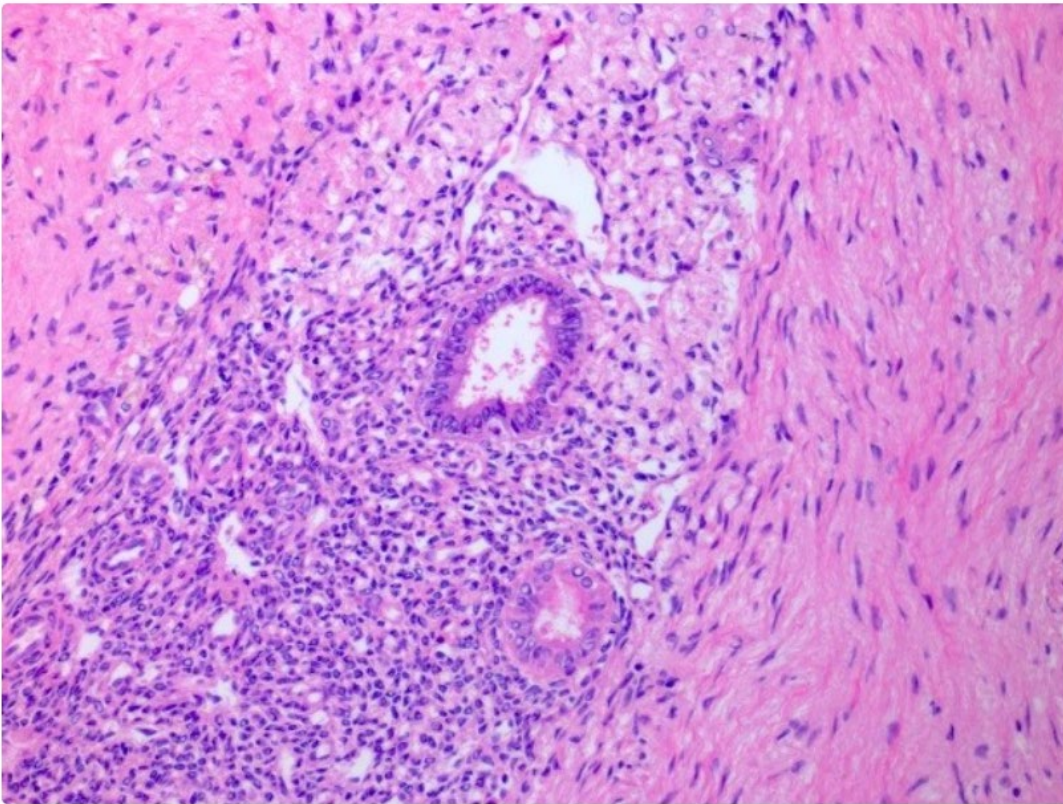
- A. Gubernaculum
- B. Processus vaginalis
- C. Genital tubercle
- D. Seminiferous cords
- E. Labioscrotal swellings

- 9) Three years later, the same boy in the previous scenario was brought to his pediatrician because of early development of pubic hair, acne, and rapid growth. Physical exam shows asymmetry of the scrotum with a firm, non-tender mass in the left testis. Serum studies show elevated testosterone but normal α -fetoprotein (AFP) and human chorionic gonadotropin (hCG). **Which of the following is the most likely diagnosis?**

- A. Seminoma
- B. Embryonal carcinoma
- C. Leydig cell tumor
- D. Yolk sac tumor
- E. Choriocarcinoma

Station 8:

- 5) A woman gives birth to a term infant via cesarean section and begins to hemorrhage. The obstetrician ligates bilateral uterine arteries. The bleeding improves and the uterus does not need to be removed. How is the uterus now receiving blood flow? Fill in the blank
collateral blood supply from ovarian arteries
- 6) A 24-year-old woman comes to the emergency department because of lower abdominal pain. A CT scan of the abdomen and pelvis shows an abnormal mass occupying the left adnexa in the pelvis. During the surgical procedure to remove the tumor, the ureter and the structures immediately medial to the ureter are identified. **Which of the following vascular structures crosses the ureter just lateral to the cervix of the uterus?**
- A. Middle rectal artery
 - B. Superior vesical artery
 - C. Internal pudendal vein
 - D. Uterine artery**
 - E. Gonadal vein
- 7) A 16-year-old woman is brought to the emergency department with severe abdominal pain and fever. Laboratory examination is remarkable for an elevated white blood cell count and a positive test for pregnancy. Colpocentesis is performed to ascertain the presence of blood in the pelvis from a ruptured ectopic pregnancy. **Through which of the following structures does the needle need to be inserted to perform colpocentesis?**
- A. Through the perineal body into the vesicouterine space
 - B. Through the posterior fornix of the vagina into the rectouterine pouch**
 - C. Through the anterior fornix into the endocervical canal
 - D. Through the introitus into the vestibular gland
 - E. Through the perineal membrane into the urogenital diaphragm
- 8) A 30-year-old woman has had recurring pelvic pain with dysuria and dysmenorrhea for the past 5 years. On pelvic examination, no abnormal findings were noted. Magnetic resonance imaging shows focal lesions in the posterior cul-de-sac along with adhesions. Histology of the lesion is provided.



Which of the following is the most likely diagnosis?

- A. Endometriosis**
- B. Hydatidiform mole
- C. Acute salpingitis
- D. Subserosal leiomyoma
- E. Poorly differentiated adenocarcinoma

5) Assemble the branches of the internal iliac artery in the 3D printed pelvis model in relation to the pelvic organs.